

other eczematouse eruptions

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Consultant dermatology

Seborrheic dermatitis



DIFFERENTIAL DIAGNOSIS OF INTERTRIGINOUS DERMATOSES IN ADULTS

Common

Irritant/frictional intertrigo*

- Ill-defined erythema/maceration
- Predisposing factors: obesity, heat & humidity, hyperhidrosis, diabetes mellitus, poor hygiene
- Secondary infections common

Seborrheic dermatitis**

- Well-demarcated, pink to red, moist patches/plaques
- Centered along inguinal creases
- Involvement of scalp, face, ears

Inverse psoriasis**

- Well-demarcated, pink to red plaques
- Shiny with little scale in folds
- Centered along inguinal creases
- Psoriasisform plaques elsewhere (e.g. genitals, intergluteal cleft, scalp, elbows/knees, hands/feet)
- Nail psoriasis (pitting, oil spots)



Dermatophytosis (tinea cruris)

- Less often centered along inguinal creases
- Expanding annular lesions with scaly erythematous border that may contain pustules or vesicles
- Extension to inner thigh, buttock; usually spares scrotum
- Coexisting tinea pedis/unguim very common



Less common

Candidiasis

- Intense erythema with desquamation and satellite papules/pustules
- Often involves scrotum as well as skin folds
- Predisposing factors: occlusion, hyperhidrosis, diabetes mellitus, antibiotic or corticosteroid use, immunosuppression



Erythrasma

- Pink-red to brown patches with fine scale
- Coral-red fluorescence with Wood's lamp illumination



Allergic contact dermatitis

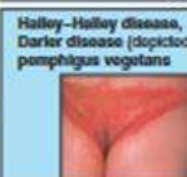
- Consider if fails to respond to usual therapy

Uncommon

Granular parakeratosis



Systemic contact dermatitis, symmetrical drug-related intertriginous and flexural exanthema, toxic erythema of chemotherapy



Halliday-Halliday disease, Darier disease (dyskeratosis), pemphigus vegetans



Zinc deficiency, necrolytic migratory erythema, other "nutritional dermatitis"



Cutaneous Crohn disease



Langerhans cell histiocytosis



Extramammary Paget disease



-
- Seborrheic dermatitis is a common mild chronic eczema affecting skin regions with high sebum production.
 - there are 2 forms of seborrheic dermatitis:
 1. Infantile form: first 3 months of life .
 2. Adult form: peaks in the 4th- 6th decades,
 - chronic adult seborrheic dermatitis is the most common type of eczema with prevalence of about 5%.
 - men are affected more common than women.

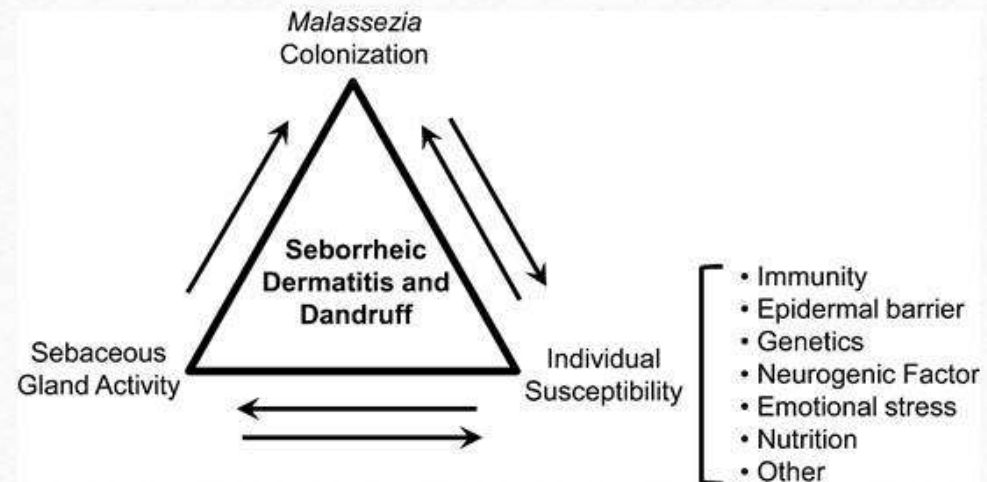
Aetiopathogenesis

I. Sebum over production :

Seborrheic area

Infant after birth

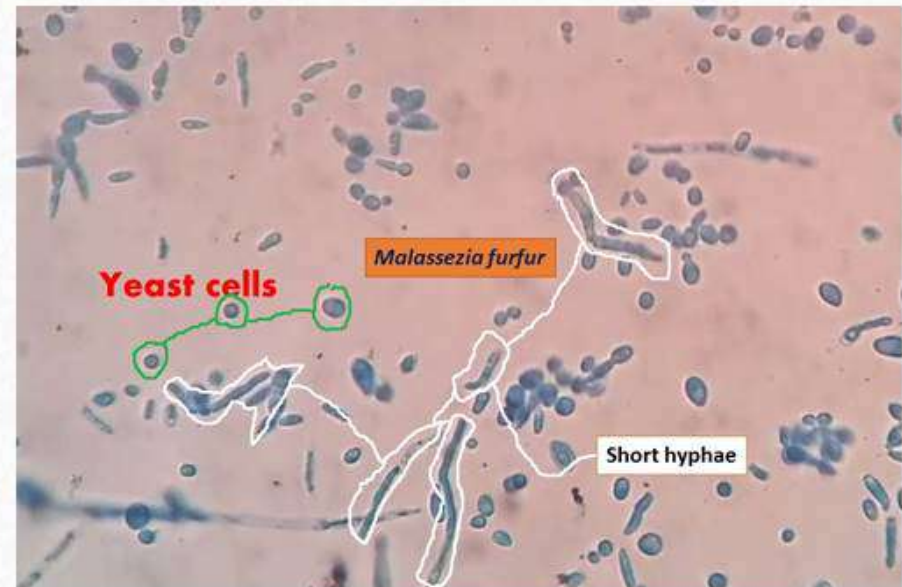
Adult after puberty



-
- Altered sebum composition
 - Increase TG , cholesterol
 - Decrease squalene , FFA
 - Decrease propionibacterium acne

- Role Malassezia

- Immune response against M FURFUR
- Irritant toxic metabolite



Clinical feature

- Red patch or thin plaque with greasy yellowish scales
- Site :sebaceous area



Clinical form

infantile

adult

1 week after birth

Types :

1-Scalp (cradle cap)

2-Trunk:flexors

3—Leiner disease

1-Scalp
Dandruff
Inflammatory
Corona
sebrhics

2-Face
Symmetric
Nasolabial
fold
Non purulent
Otitis media

3-Trunk
Petaloid
Flexours
Eczematus
ptyrasiform

4-Generalized
5-Erythrodermic



Leiner's disease (Erythroderma desquamativum):



a severe, widespread, erythrodermic form of infantile seborrheic dermatitis.

- * fever, anemia, diarrhea, vomiting, weight loss, and sometimes death
- a) Non-familial
- b) Familial : deficiencies of complement C3,C5, and phagocyte malfunction



Figure 2 Exfoliative erythroderma in a neonate with Netherton syndrome. In

Treatment:
intense IV hydration, temperature
regulation, and antibiotic.
Patients with the hereditary : fresh
frozen plasma

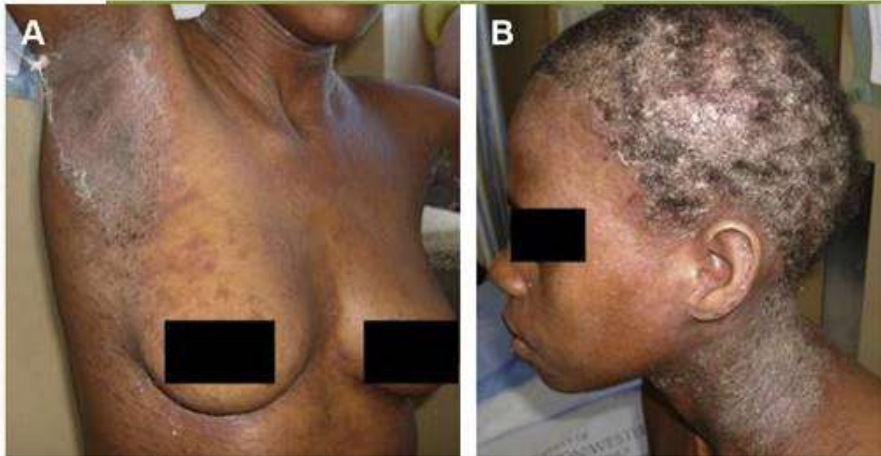




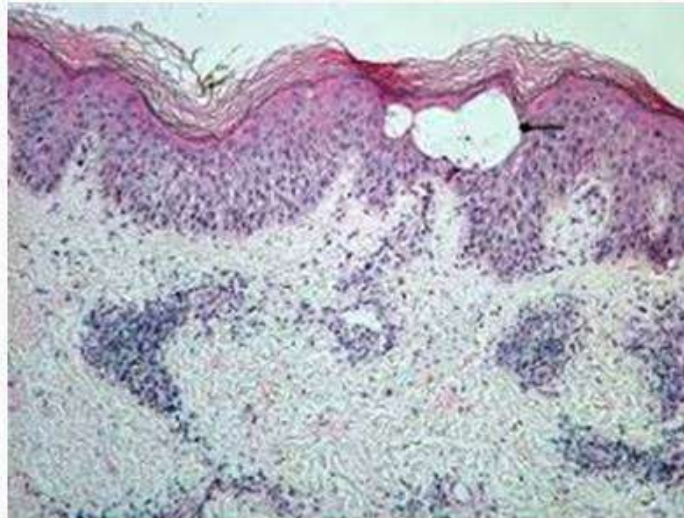
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Association



- HIV : extensive and severe seborrheic dermatitis may be a sign of underlying HIV infection
- Parkinson disease



- Histeopathology:
- Spongiotic dermatitise

Treatment

no perminant cure , repound

General skin
care
Emojent
shampoo

Topical
antifungal
Ketoconazol

Topical anti-
inflammatory
Topical steroid



Steatotic eczema

winter eczema , winter itch

pathogenesis

Exogenous
Dry climate
Diuretics
Cimetidine
Soap , water

Endogenous
Aging
Malnutrition
hemodialysis



- C/P : xerosis itching, stinging
- Dry dull skin with fine bran like scales
(dry river)
- Shin of tibia , thigh
- Proximal extrematuose , trunk
- Spare face , neck , palm , sole .
- DD: atopic dermatitis , allergic contact dermatitis
- TT : emollient , topical steroid ointment



eFig. 13.3 Asteatotic eczema (eczema craquelé). **A** The distal lower extremity has areas of dull erythema that contain a criss-cross pattern of superficial cracks and fissures said to resemble a dried river bed. **B** A close-up of the pattern.

Venous eczema (stasis dermatitis)

- Component of chronic venous insufficiency of LL
- venouHTN >>> slow blood flow >>>> damage of capillary >>>> increase permeability
- Passage of fluid >> oedema
- Passage of protein>> hyaline cuff
- Extravasation of RBCS , hemosedrine



Fig. 13.7
Autosensitization
dermatitis in a patient
with venous ulceration.
Note the involvement of
the extensor surfaces of
the upper extremities in
this patient with an
allergic contact
dermatitis to neomycin,
as well as stasis
dermatitis and venous
ulceration. Courtesy, Ivan I
Rokhina, MD



c/p

Venous insufficiency :5B

Pitting oedema

Purpra

Dry itchy skin

Firm circular cuff

Inverted wine bottle

Hemosidrine pigment

Atrophic blanch

Venous ulcer

Stasis dermatitis

Sever pruritis

Lichenification

Medial malleoli



- H/P :acute , chronic , subacute
- DD : shamberg disease , pseudokaposi
- TT :tt of venouse insufficiency
- steroid , emolient

Ptyriasisi alba



- Low grade eczem
- Minor feature of atopic dermatitis
- Possiple factor:
 - Sun exposure
 - Frequent hot path
 - Decrease serum copper
 - Abnormal transfer of melanosome
- c/p: ill defined hypopigmented macule or patch, with fine scales
- Face , trunk



- DD: PIH , TVC , vitiligo , plc , mf
- TT: sun screen
 - Photoprotection
 - Emolient , topical steroid

Discoid Eczema nummular dermatitis



eFig. 13.4 Nummular dermatitis.

oid oid disease ,sulc berger grabe)

- Coin shape: uncommon
 - c/p: intense pruritis / extremitres
- .lower leg >.Men . forarm, hand >>..women

Discoid eczematus patch , vesicle ,
lichenification

Very chronic course

Therapy resistant : high , medium potency
steroid



DD :SUPERFICIAL SCALY PLAQUE:

- Eczema
- Tinea
- Bown's disease
- Paget disease
- ps



eFig. 14.15 Airborne contact dermatitis. Note the significant involvement of the eyelids. *Courtesy, Joyce Rico, MD.*

- EYE LID ECZEMA :

- Atopic eczema
- Seborrhic
- Contact eczema
- Idiopathic eczema

Hand eczema



classification

exogenous

1 -Contact irritant
Chemical(soap ,
detergent
Physical (friction ,
cold ,

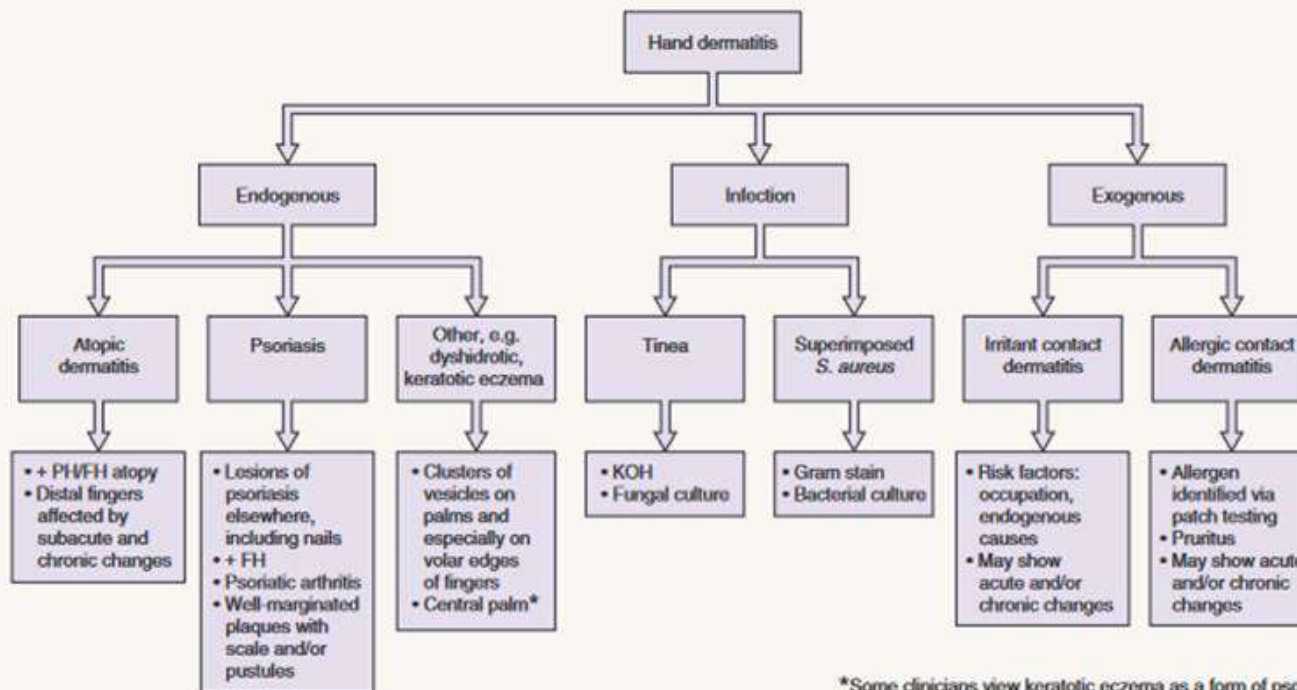
2-Contact allergen
Delayed hypers-
Immediate hypers-

3- ingested allergens:(drug , nickel
4-Infections
5- autoimmune reaction

endogenous

1-Idiopathic
2-Immuno -(atopy)
3-Psychological
4-Dyshidrosis

CLASSIFICATION OF HAND DERMATITIS



*Some clinicians view keratotic eczema as a form of psoriasis.

Patteren

A P C D E F G H

1. Apron eczema : proximal palmer aspect of 2 adjacent finger (endogenous)
2. Pomphilyx
3. Chronic acral dermatitis
4. Discoid eczema
5. Finger tip eczema : occupational dominant hand
6. Gut eczema : slaughter house , finger web
7. House wife eczema (exogenous)
8. Hyperkeratotic eczema : scaly fissured , hyperkeratotic patch
9. Ring eczema





Pompholyx dyshidrotic eczema

Special form of hand eczema (palm , sole)

Deep seated vesicles , bullae

Causes :SHAAADI

- Hereditry
- Atopic dermatitis
- Allergic contact dermatitis
- Aid reaction
- Drugs
- Stress , sun



eFig. 13.6 Dyshidrotic eczema. Firm papules and pseudovesicles on the palms and volar sides of fingers. Annular collarette-like scaling in the dyshidrosis lamellosa sicca (keratolysis exfoliativa) variant.

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SYMMETRIC FIRM DEEP SEATED VESICLES

Highly pruritic

Resolve by desquamation

H.P

- Spongiosis with micro ,macrovesicles
- TT: search for the cause , topical , systemic steroid



- DD :
 - Tinea manum
 - Tinea pedis
 - Ps
 - Dyshidrosiform pemphigoid

Lichen simplex chronicus



- Chronic eczema
- Single heavily lichenified plaque
- Severe pruritis > severe scratching > lichenification >
- Cause
 - Stress
 - Atopy
 - racial



- Site
 - Nap of neck
 - Vulva , scrotum
- TT
 - Psychological history
 - Sedation
 - Sedative anti histaminic
 - V. potent topical steroid

Juvenile planter dermatosis



Fig. 13.10 Juvenile plantar dermatosis in a child. Erythema and scaling of the plantar surface of the great toes and fifth toes as well as the ball of the foot bilaterally. Note the glazed appearance of the skin of the left foot. *Courtesy, Kalman Watsky, MD.*

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- Glazed thin skin , xerosis, cracks
- Prepubertal male
- Winter
- Non porous socks , shoes

Contact dermatitis

- Special type of eczema that occurs when skin come in contact with sexternal agent.
-

Irritant CD

Allergic CD

Photocontact dermatitis



Fig. 14.10 Allergic contact dermatitis due to cashew nut shell oil. This represented an occupational exposure.





Fig. 14.6 Acute allergic contact dermatitis with a prominent component of edema. **A** Periorbital edema, in addition to crusted and weeping plaques, due to poison ivy. **B** Allergic contact dermatitis to methylisothiazolinone in a wet wipe used to remove make-up. *A, Courtesy, Jean L. Bolognia, MD.*

PATTERNS OF ALLERGIC CONTACT DERMATITIS

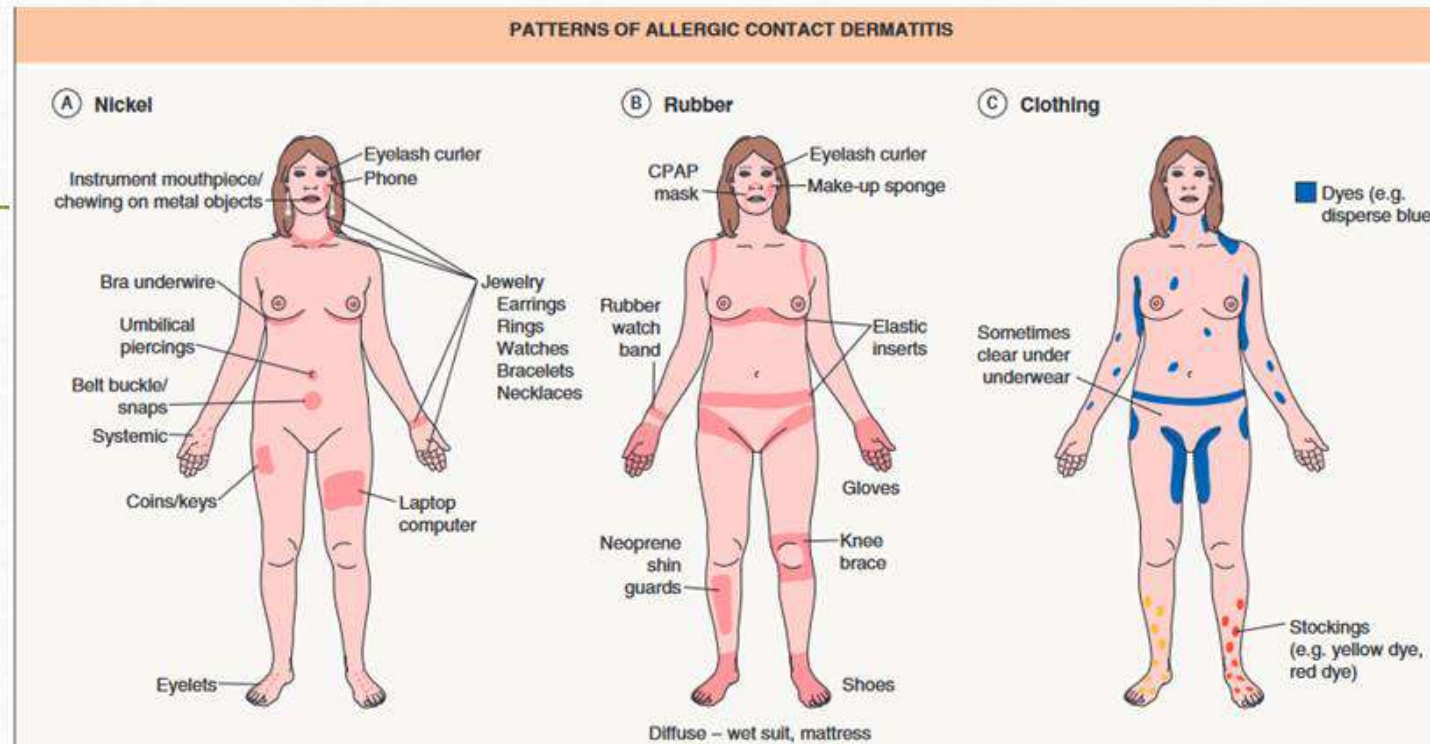


Fig. 14.9 Patterns of allergic contact dermatitis. **A** Due to nickel; lesions can also arise from metal braces and on the back at the site of bra clasps. **B** Due to rubber. **C** Due to dyes and resins in clothing. CPAP, continuous positive airway pressure

	Irritant contact dermatitis	Allergic contact dermatitis
	80%	20%
	Irritant chemicals (soap , solvent , alkalis , acids)	Allergens :nickel , fragrances , neomycin, poison ivy, bivalirudin
	Local toxic effect > damage to skin > barrier disruption > inflammation	Delayed type 4 hypersensitivity reaction • Initial sensitization phase
	Occur in all subjects	Only in subject sensitive to allergens
	Previous contact not required	Sensitization is necessary

Skin lesions	Acute	sub acute	chronic
	At site of exposure	At site and away from	
treatment	Avoid irritant , triggers , emollient , topical steroid		

Patch test

HOW TO : SKIN PATCH TEST

- | | |
|--|--|
| 1
Choose the Skin Part

1. Inside of Your Elbow 2. Inside of Your Knee 3. Neck Below Your Ear | 2
Clean and Make Sure It Dries Completely
 |
| 3
Apply a Tiny Amount and Spread It
 | 4
Apply a Bandage, Wait for 24 hours
 |

- Simple test for diagnosis of allergic contact dermatitis
- Detect type 4 hypersensitivity reaction

- Dilution of common allergen to back under aluminium discs >>>
- Left for 48 h>>>



eFig. 14.7 Allergens contained within syringes being placed by nurse into Finn chambers.



eFig. 14.8 Fixing allergens to patient's back using Scanpor® tape.



eFig. 14.9 Allergens being marked upon removal of Scanpor® tape.



- Interpretation :

1. No reaction
2. Minimal erythema (doubtful reaction)
3. Erythema may be popular (+weak reaction)
4. Erythema , oedema ,may vesicular (strong reaction ++)
5. Erythematuse ,bullouse (+++extrem reacriion)
6. Irretant reaction (sharply circumscribed with glased appearance)

INTERNATIONAL GRADING SYSTEM FOR PATCH TESTS	
+/-	Doubtful reaction, faint macular erythema
+	Weak, non-vesicular reaction with erythema, infiltration and papules
++	Strong, vesicular reaction with erythema, infiltration and papules
+++	Spreading bullous reaction
-	Negative reaction
IR	Irritant reaction

Table 14.4 International Grading System for patch tests. See Fig. 14.14.

ROAT TESI

- Repeated open application test

